

copy No. 6

Date:

Examination: Histopathological examination

Internal invoice No.

Value of diagnostic procedure

Examination No.:

Patient: XXX

Age: Gender: F

Material: Total organ resection - ~~Right breast~~

Unit in charge: Central Operating Theatre

Physician in charge:

Material collected on:

Material received on:

Expected time of examination: 5 working days

Clinical diagnosis:

1CB-0-3
 path: carcinoma, infiltrating duct end mucinous 8523/3
 CQCF: "other" carcinoma, ductal, papillary 8503/2
 Site: breast, NOS C50.9 for 4/12/11

Examination performed on:

Macroscopic description:

Right breast sized 21 x 17 x 5 cm removed without axillary tissues with a skin flap of 19 x 8.5 cm.

Weight 700 g.

Tumour sized 2.2 x 1.9 x 2.4 cm in the upper outer quadrant, located 2.1 cm from the upper boundary, 0.7 cm from the base and 1.5 cm from the skin.

Microscopic description:

Carcinoma mucinosum partim ductale invasivum - NHG2 (3 + 2 + 1/21 mitoses/10 HPF - visual area 0.55 mm).

Foci of carcinoma ductale in situ DCIS found within the tumour (solid and cribrate type, with medium nuclear atypia and comedo necrosis 2% of the tumour).

Lesions in situ of the type carcinoma intraductale mamillae.

Lesions in situ of the type mastopathia fibrosa et cystica in the glandular texture.

AXILLARY LYMPH NODES

Sinus histiocytosis in lymphonodo (No I).

Test result

Final diagnosis to be given after examination

Compliance validated by:

Examination performed on:

Results of immunohistochemical examination:

Estrogen receptors found in over 75% neoplastic cell nuclei. Progesterone receptors found in over 75% neoplastic cell nuclei.

HER2 protein stained with by . Negative reaction in invasive cancerous cells (Score = 0)

Compliance validated by:

Examination performed on:

 UUID: D3EA3547-7E53-442E-A957-B2DD7EBAFB89
 TCGA-D8-A1X7-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary		☒
Case # (Circle):	QUALIFIED	DISQUALIFIED
Reviewed by:	Initials	Date Reviewed:

for 4/12/11

Examination: Histopathological examination

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Examination No.:

Patient: XXX

Gender: F

Examination performed on

Histopathological Diagnosis:

(Including examination

Carcinoma mucinosum partim ductale invasivum et ductale in situ mammae dextrae. Invasive mucinous and in part ductal carcinoma. and ductal in situ carcinoma of the right breast.
(NHG2, pT2, pN0/sn/).

Compliance validated by:

CONTACT YOUR DOCTOR WITH THIS REPORT!

TCGA Pathologic Diagnosis Discrepancy Form

V4.00

Instructions: The TCGA Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for TCGA is inconsistent with the diagnosis provided on the Case Quality Control Form completed for the submitted case.

Tissue Source Site (TSS): _____ TSS Identifier: _____ TSS Unique Patient Identifier: _____

Completed By (Interviewer Name): _____ Completed Date: _____

Diagnosis Information			
#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	"Ductal with mucinous"	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for TCGA, as reflected on the CQCF.	"Ductal partially papillary"	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form			
3	Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form.	Discrepancy probably due to methodology of slide preparation	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
4	Name of TSS Reviewing Pathologist or Biorepository Director	_____	Provide the name of the pathologist who reviewed this case for TCGA.

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

TSS Reviewing Pathologist or Biorepository Director Date

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

Principal Investigator Signature Date